

Name: مجده حامد حسين يعقوب
National ID: 27610132102222



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



Name : Majda Hamid Hussein Yaqub

Date : 26-03-2026 06:56

Branch : Dokki

Medical No. : 336103

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاليد مستشفى

Whole Body F18- FDG PET/CT study

Clinical history:-

50 year old female, presenting with surgically resected left breast cancer received chemotherapy, currently presenting with left hepatic lobe focal lesion and referred for PET CT evaluation.

Procedure:

- ✗ Radiopharmaceutical: 18F FDG, 3.8 mCi I.V.
- ✗ Blood glucose level at time of 18F-FDG injection: 317mg/dL .
- ✗ Time from 18F-FDG injection to scan: 60 min
- ✗ Technique :MDCT diagnostic examination was taken for attenuation correction anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- ✗ Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.
- ✗ Patient's height: cm patient's weight: 75kg
- ✗ SUV average of reference hepatic activity=2.5.
- ✗ Measurements are correlated to body weight.



PET/CT FINDINGS:-

Head and Neck:

- No metabolically active cervical or supra-clavicular LNs noted.
- The brain exhibited normal FDG bio-distribution
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Normal thyroid gland FDG uptake with no focal lesions.

Name: <u>مجده حامد حسين يعقوب</u>		
MR. : <u>8950</u>		
Detailed Medical Report		
		السيد
تخية طيبة وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه		
A 49-year-old diabetic patient underwent left simple mastectomy with sentinel lymph node biopsy, resulting in a T1c N0 staging. The tumor was ER-negative, PR-negative (0/8), and HER2-positive (+3). Bone scan was negative; CT of the chest, abdomen, and pelvis and MRI abdomen were unremarkable. A hemangioma was noted in the hepatic focal lesion (HFL) on imaging. A PET-CT was pending prior to initiation of adjuvant therapy. The multidisciplinary team recommended starting Taxol (paclitaxel) for 12 weeks plus Herceptin (trastuzumab) for 6 months. The patient received the first dose of Taxol (80 mg/m², 145 mg) in week 1, with laboratory values within acceptable limits. Subsequent doses were administered weekly: week 2 (145 mg), week 3 (145 mg), week 4 (145 mg), week 5 (145 mg), week 6 (145 mg). Herceptin was still pending at that time, and a PET-CT was awaited to guide further management. In week 7, the patient was vitally stable with a performance status of I but reported grade II-III peripheral neuropathy. The dose of Taxol was reduced to 60 mg/m² (108 mg) after laboratory review, and supportive therapy was initiated. The same dose was administered in week 8, with the patient's weight at 75 kg, height 160 cm, and BSA 1.8 m². By week 10, the patient experienced severe peripheral neuropathy; methylprednisolone was added to the regimen while awaiting laboratory results. In weeks 11 and 12, the patient sustained a finger injury during a surgical procedure on the arm, prompting referral for surgical consultation and initiation of Augmentin and alphantern for infection prophylaxis. In July, the patient's diabetes remained uncontrolled, with an elevated HbA1c. The patient received a third dose of Herceptin SC and was scheduled for an echocardiogram at the next visit.		
تحريراً في ,		
التاريخ :	20-09-2026	الوقت :
الطبيب المعالج :	هنا أسامه عبد الرحيم عبد البر	
يعتمد ,		

Name: Majda Hamid Hussein Yaqub

Date: 26-03-2026 06:56

Branch: Dokki

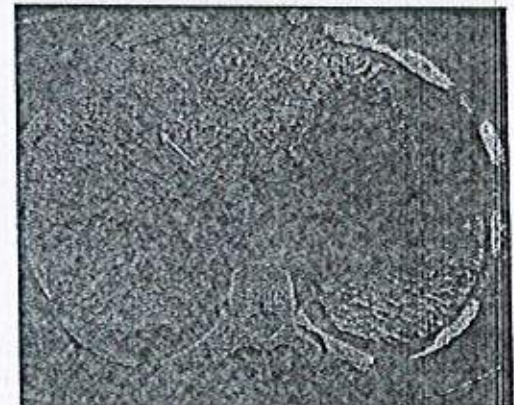
Medical No.: 336103

Ref. Physician Prof: Goustav Roussey Egypt

Ins. Company: تعاقد مستشفيات

Chest:

- Clear left mastectomy operative bed with no FDG avid lesions to account for recurrent neoplasia.
- No metabolically active right breast lesions.
- No metabolically active mediastinal, axillary or hilar lymph nodes noted.
- Normal FDG uptake is seen throughout both lungs with no FDG avid lung nodules or masses detected. A tiny non-FDG-avid subpleural 2 mm pulmonary nodule is seen at the right lung upper lobe anterior segment.
- There are no pleural or pericardial abnormalities.
- Physiologic FDG uptake is noted in the myocardium.



Abdomen and Pelvis:

- The liver shows a non-FDG-avid segment IVa hypodense focal lesion measuring 15 x 14 mm.
- The stomach shows non-FDG-avid mural thickening along the greater curvature achieving SUV max of 21 mm.
- No evidence of hepatic or splenic FDG avid focal lesions.
- No metabolically active pelvi-abdominal LNs noted. Bilateral inguinofemoral non-FDG-avid reactionary looking lymph nodes.
- There are no FDG avid adrenal nodules.
- No evidence of FDG avid peritoneal nodules
- Physiologic FDG excretion is seen in the kidneys and bladder.
- No evidence of ascites.

Name: Majda Hamid Hussein Yaqub

Date: 26-03-2026 06:56

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Ref.Physician Prof: Goustay Roussey Egypt

Ins.Company: معاهد مستشفيات

Musculoskeletal:

- Normal FDG uptake is noted at the surveyed skeleton with no FDG avid osteolytic or sclerotic lesions.
- Spondylodegenerative changes of the surveyed spine.
- Left shoulder joint active osteoarthritis.

Conclusion:

First PET/CT study revealed:

- No evidence of recurrent breast neoplasia.
- Non-FDG avid hepatic focal lesion as described above, equivocal in nature for correlation with tumor markers and close follow-up.
- Non-FDG-avid nonspecific tiny pulmonary nodule for follow-up.

Thanks for referral;
Dr. Heba Elnaggar, MD
Lecturer of Radio-diagnosis,
ASUH

MDT FORM

17/12/2025 G

Name	ميرة ن4	MRN	
Age	49	ID	CV71-121-CCC
Menopausal status	Premenopausal	Phone	0112210957
NRS SCORE		Marital status	Married
BMI		Family history	Nil
CO morbidity	DM	REASON OF MDT	Systemic therapy

Clinical Data

BX: IDC G2				R	ptic No	L
ER:	PR:	HER 2:	Ki67:			
0/8	0/8	3+				
Sonomammography:						
Left simple mastectomy + SLNB				TNM for MRI (mostly benign)		
Mets Workup:						
CT CAP → HFL Bon Scan: Free						

Recommendations:

Adj Herceptin + Taxol weekly x12 weeks

Path review & MRI L (R)

MDT OUTCOME & PLAN

ptic No for adj. herceptin + taxol 12 weeks

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
		New Pm 12/11/2025			

Patient Name : الأستاذ / موجه حامد حسين يعقوب	Request No. : 13003724
Patient No. : 27610132102222	Request Date : 30/11/2025
Path No. : SR-1733/25	Verification Date : 15/12/2025
Sex : Female	Financial Category : جوستاف روسي - لفقة الدولة
Age : 49 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data

Left breast mass.

Nature of specimen

Block & slide revision .

Gross examination

15 paraffin blocks (6377-25) & 7 slides were received .

Microscopic examination

- * Histological type: Multifocal Invasive duct carcinoma, LARGEST TUMOR FOCUS : (4mmx4mm)
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
- * DCIS: Intermediate grade , solid , cribriform , subtypes with comedo necrosis
- * Lobular intra epithelial neoplasia : Absent
- * Angioinvasion: Absent .
- * Microcalcification : Absent.
- * Perineural invasion: Absent
- * Deep margin : Free.
- * Nipple & areola : Free of tumor tissue or paget's disease .
- * Lymph nodes : Free (0/6)
- * IPT :
- * ER: No nuclear reaction in tumor cells (Allred score 0/8)
- * PR: No nuclear reaction in tumor cells (Allred score 0/8)
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (3+)

Diagnosis

LEFT BREAST MASS, BLOCK & SLIDE REVISION:

-Multifocal Invasive duct carcinoma with microcalcifications Grade 2 (tubular differentiation

Verified by :

Dr.Heba Hasan



Patient Name	: الأستاذ / مجده حامد حسين يعقوب	Request No.	: 13003724
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Age	: 49 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

2, pleomorphism 2, mitosis 2) (according to Nottingham grading system of breast cancer)
, with intermediate grade DCIS , solid , cribriform subtypes . with comedo necrosis
& calcification

-Free nipple of paget's diseases or tumor tissue.

-Free deep margin .

_Free lymph nodes (0/6) .

IPT:

* ER: Negative (score 0/8).

* PR: Negative (score 0/8).

* HER2: Positive (score +3)

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof Dr.Maha Guimei

Verified by :

Dr.Heba Hasan



PATIENT NAME: MOGADA HAMED HUSEN YAAKOUB

GENDER: Female

DATE : 18-Nov-2025 02:26 PM

PATIENT ID: 20225023950

AGE: 49 y

ORDER: 3643910,3643911

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN
AND PELVIS WITH TRIPHASIC STUDY OF THE LIVER:****FINDINGS:****1-CHEST**

- Left mastectomy operative bed ill-defined hypodense sheets intermingled with localized collection measuring about 9.6 x 1.4 cm. Associated overlying edematous skin thickening of about 8 mm with air loculi.
- Another operative bed collection is related to lateral aspect of left pectorals major muscle, measuring about 3.2 X 1.9 cm.
- No sizable operative bed enhancing mass lesions.
- Few subcentimetric left axillary level I & II lymph nodes, the largest at level I measuring about 6 X 7 mm.
- Right upper lobe anterior segment subcentimetric pulmonary nodule measuring about 4 mm.
- No pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild increase cardiac size.
- No pericardial effusion detected.
- Bilateral elastofibroma dorsi more on the right side.

2- ABDOMEN AND PELVIS

- Enlarged liver showing segment IVa/VIII hypo-enhancing focal lesion measuring about 1.3 X 1.6 cm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

PATIENT: MOGADA HAMED HUSEN YAAKOUB, 20225023950, Female, 49 y

Report version 2, 19-Nov-2025 08:04 PM

Page 1 of 2



- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, adrenals and kidneys " apart from left perirenal cysts".
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Mild distended uterine cavity by hypodensities, for clinical correlation.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- Mild pelvic ascites.
- **The revised bone window revealed:**
 1. No CT evidence of vertebral collapse or extra-osseous soft tissue component.

Opinion:

- ***Left mastectomy operative bed ill-defined hypodense sheets intermingled with localized collections as above described.***
- ***Right upper lobe subcentimetric pulmonary nodule, for follow up.***
- ***Hepatic segment IVa/VIII hypo-enhancing focal lesion, for further dynamic MRI assessment.***

Reported By,
Dr. Sara Yousry, MSc
Dr. Amr Farouk, MD

19-Nov-2025 08:04 PM

PATIENT: MOGADA HAMED HUSEN YAAKOUB, 20225023950, Female, 49 y

Report version 2, 19-Nov-2025 08:04 PM

Page 2 of 2

PATIENT NAME: MOGADA HAMED HUSEN YAAKOUB
GENDER: Female
DATE : 16-Nov-2025 11:39 AM

PATIENT ID: 20225023950
AGE: 49 y
ORDER: 3643912

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

17-Nov-2025 02:31 PM



PATIENT NAME: MOGADA HAMED HUSEN YAAKOUB

GENDER: Female

DATE : 30-Sep-2025 03:00 PM

PATIENT ID: 20225023950

AGE: 48 y

ORDER: 2030115

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY**INDICATION:**

-Left breast UOQ focal asymmetry with altered parenchyma and suspicious masses along with ipsilateral indeterminate axillary LN

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views
- IV non-ionic contrast medium infusion 100 ml of omnipaque

REPORT:

- Contrast Enhanced Spectral Mammography showed bilateral minimal symmetrical back ground parenchymal enhancement .
- Right inner and retroareolar faint enhancement noted on CC views only (early) with no mammographic correspondence likely adenotic ..
- No right breast suspicious mass or non mass enhancement.
- Left UOQ segmental non mass moderate heterogeneous enhancement extending to nipple and measuring 106x52x97mm.
- Normal skin thickness and contour.
- Normal nipple areola complexes.

OPINION:

Left UOQ non mass suspicious enhancement (BIRADS4c).

- No Right breast pathological mass or non mass enhancement (BIRADS2)

Reported By,

Dr. Rehab El sheikh, MD

30-Sep-2025 04:10 PM



PATIENT NAME: MOGADA HAMED HUSEN YAAKOUB
GENDER: Female
DATE : 30-Sep-2025 03:00 PM

PATIENT ID: 20225023950
AGE: 48 y
ORDER: 2

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY

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Reported By,

Dr. Rehab El sheikh, MD

30-Sep-2025 04:10 PM